

Objectives

1. Students will understand the principles of ART and MFR and their applications.
2. Students will understand the indications and contraindications of ART and MFR.
3. Students will understand the basic technique principles of ART and MFR.
4. Students will understand the principles of regional vs. segmental vs. peripheral ART and their applications.
5. Students will understand the models of MFR and their applications.

ART Basic Technique Principles:

1. The patient should be comfortable and able to relax.
2. The physician should also be in a position of comfort to minimize energy expenditure and to make use of body weight and leverage rather than having to exert muscular force.
3. The physician moves the affected joint or body part through its range of motion until the restrictive barrier is engaged.
4. A gentle but firm force is applied, carrying the body region a short distance through the restrictive barrier. The distance is limited by the tissues being treated and the patient's tolerance to pain or fatigue.
5. This force is typically applied rhythmically, consisting of 1 or 2 seconds of stretch followed by a similar time frame releasing the stretch. The joint is permitted to return to a point just short of its restrictive barrier.
6. As the patient responds to the technique, the restrictive barrier will shift position within the physiologic range of motion. For each cycle of the applied technique, re-engage the restrictive barrier and carry the affected body region a short distance farther, through the new barrier to normal motion.
7. The applied forces should be comfortable to the patient. Some patients experience discomfort, but it is recognized by the patient as a good and necessary discomfort.
8. The technique is continued until the location of the restrictive barrier reaches a plateau; that is, no further increase in range of motion can be achieved by continuing the technique or until full physiologic range of motion has been restored to the joint(s) being treated.
9. At no time is the anatomic barrier to joint motion exceeded.

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